



JDScience BTEC Health and Social Care Unit 2 Set A mark scheme

Original JDScience mark scheme. Indicative content only. Equivalent wording that meets the command word should be credited. Not an official Pearson paper.

Use this booklet after the matching practice paper. Do not issue it with the student paper.

Time: 1 hour 30 minutes · **Total marks:** 80

Question 1 · 20 marks

Oakfield is a mixed urban area. Residents use the GP surgery, a community pharmacy, the local authority adult social care team, a hospital 8 miles away, a British Red Cross loan service for wheelchairs, and a small private homecare agency. Informal carers include neighbours and adult children.

(a) [2 marks]

Identify two organisations that provide primary care in this case.

- GP surgery; community pharmacy. (Hospital is secondary/tertiary — do not award for hospital as primary.)

(b) [4 marks]

Describe the role of the local authority in adult social care.

- Assess needs under the Care Act; arrange or provide care and support; safeguarding adults; information and advice; may charge for some services.
- Working with NHS and voluntary sector; not replacing GPs.

(c) [6 marks]

Explain the difference between statutory, voluntary and private provision using Oakfield examples.

- Statutory: GP/NHS hospital/local authority — legally required, publicly funded.
- Voluntary: Red Cross — not-for-profit, often donations/volunteers, fills gaps (wheelchair loan).
- Private: homecare agency — paid by individual or local authority contract, profit-making.
- Must use the case, not generic lists only.

(d) [8 marks]

Discuss how informal carers and formal services can work together to meet residents' needs.

- Informal: daily help, knowing the person, but risk of strain and no training.
- Formal: clinical/social care skills, regulation, can give informal carers a break.
- Partnership: sharing information with consent; carer's assessment; not assuming family will cope.
- Level 3: balanced discussion with a conclusion about better outcomes when both work together.

Question 2 · 20 marks

Amira is 82 and has type 2 diabetes and reduced mobility. She sees her GP, a practice nurse for reviews, a podiatrist, and a social worker after a fall. A support worker from the private agency helps her with meals. Amira's son lives 200 miles away.

(a) [2 marks]

Identify two health professionals in this case.

- GP; practice nurse; podiatrist. (Social worker is social care; support worker is care/support.)

(b) [4 marks]

Outline the role of the social worker after Amira's fall.

- Assess care and support needs and risks; consider reablement, homecare, equipment; safeguarding if neglect/harm suspected; involve Amira in decisions.

(c) [6 marks]

Explain how two professionals could work together to support Amira.

- GP/nurse share diabetes and mobility information (with consent) so the social worker plans realistic care.
- Podiatrist and support worker: foot care and prompting medication/meals reduce further falls.
- Multidisciplinary communication, not working in silos.

(d) [8 marks]

Evaluate possible barriers Amira may face when accessing services.

- Physical: mobility, transport 8 miles to hospital.
- Geographical/social: son far away — less informal advocacy.
- Psychological: pride, fear of losing independence.
- Financial: private homecare charges if not funded.
- How barriers might be reduced (transport, advocates, financial assessment).
- Level 3: range of barriers applied to Amira with a judgement.

Question 3 · 20 marks

The Hayes family includes Jamal, 9, who has a moderate learning disability and epilepsy. He attends a mainstream school with a teaching assistant. His mum wants short-break care. Jamal becomes distressed in noisy waiting rooms.

(a) [2 marks]

Identify two settings that may support Jamal.

- Mainstream school; GP/paediatrics; children's social care; short-break/respite; community learning disability team.

(b) [4 marks]

Describe two ways staff can communicate effectively with Jamal.

- Simple language, extra time, visual timetables, social stories, involving mum as he wishes, quiet space instead of noisy waiting rooms.

(c) [6 marks]

Explain why a person-centred approach is important when planning Jamal's care.

- Jamal's preferences, strengths and epilepsy needs, not a standard package.
- Mum as expert on his distress in noise — environment should adapt.
- Dignity, choice and better engagement/safety (medication, seizures).

(d) [8 marks]

Discuss how health, education and social care could work together for Jamal.

- EHCP/school TA, community paediatrician/epilepsy nurse, social care short breaks.
- Information sharing with consent; joint reviews.
- Challenges: different systems, waiting times; benefits of one plan.
- Level 3: applied discussion with a conclusion.

Question 4 · 20 marks

A homecare worker notices unexplained bruises on an adult with dementia and that food has been left uneaten. The family ask her not to "make a fuss". The agency has a safeguarding policy, a confidentiality policy and a whistleblowing procedure.

(a) [2 marks]

Identify two care values the worker should show.

- Examples: promoting dignity; safeguarding; honesty; person-centred care; respect; not keeping harmful secrets.

(b) [4 marks]

Describe the worker's safeguarding responsibilities in this situation.

- Recognise possible abuse/neglect; record facts; report to the manager/safeguarding lead without investigating alone; do not promise secrecy to the family.

(c) [6 marks]

Explain how confidentiality still applies, and when information must be shared.

- Day-to-day personal information stays private (GDPR/Data Protection, Caldicott principles).
- Sharing on a need-to-know basis with the safeguarding lead is required if there is a risk of harm — confidentiality is not absolute.
- Family request does not override duty to report.

(d) [8 marks]

Evaluate the importance of policies and whistleblowing for safe practice.

- Policies give a clear route so workers are not left to decide alone.
- Whistleblowing protects people who use services if internal reporting is blocked, and protects the worker who raises concern in good faith.
- Limits: policies only work if trained and used; fear of family conflict.
- Level 3: judgement that policies are essential but culture and training make them effective.